AM-04 Hypertension / Suspected Hypertensive Emergency

MEDICAL-DIRECTOR APPROVED APP CONTENT | SOURCE-PDF REPLACEMENT | Reconciled October 16, 2026

Scope: Use for markedly elevated BP with suspected acute end-organ injury. Treat the condition causing the elevation, not an isolated number.

Targeted blood-pressure evaluation and treatment

1. Confirm BP	Correct cuff size. Repeat an extreme or inconsistent automated value manually when feasible.
2. End-organ emergency?	CVA/stroke, ACS, pulmonary edema, aortic syndrome, encephalopathy, pregnancy-related symptoms, or reduced perfusion.
3. Undifferentiated CVA	If manually confirmed SBP is above 220 or DBP is above 110, give labetalol 10 mg slow IV over 2 min.
4. Reassess	Every 5 min: BP, HR, rhythm, perfusion, and focused neurologic exam.
5. Still above target?	After at least 10 min, repeat labetalol 10 mg once only if still SBP above 220 or DBP above 110. Maximum total 20 mg.
6. Goal BP	SBP 220 or less and DBP 110 or less. Do not lower further without a condition-specific order.
7. Other acute injury	Use linked ACS, pulmonary-edema, aortic, obstetric, or transfer pathway. Medical Control defines non-stroke antihypertensive treatment.
8. No acute injury	Address pain, anxiety, hypoxia, urinary retention, stimulant exposure, or missed medications. Do not give antihypertensive medication solely to normalize BP.

Key treatment and disposition

Item	Approved action
Labetalol exclusions	Do not give for pulmonary edema, acute decompensated heart failure, cardiogenic shock, bradycardia, advanced heart block, or active bronchospasm.
TNK/post-thrombolysis	Do not apply the 220/110 undifferentiated-CVA target. Use AM-06 and written sending/receiving-facility orders.

Do not let BP treatment delay stroke alert, definitive imaging, or time-critical transport.

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SAFETY, LINKED PATHWAYS, AND RELEASE CONTROL

Safety checks

- Rapid, non-targeted BP reduction can reduce cerebral, coronary, and renal perfusion.

- A suspected stroke is undifferentiated until imaging. Do not administer an antihypertensive solely to normalize BP.

- Do not administer a patient's home antihypertensive unless specifically authorized.

- For any non-stroke hypertensive emergency, Medical Control must define the indication, dose, and endpoint unless a linked condition-specific protocol explicitly authorizes treatment.

Linked pathways

Pathway	Use when
UP-14	Suspected stroke assessment, alert, destination, and matching CVA BP branch
AM-06	Post-TNK transfer; written facility orders govern
AC-04	ACS or ischemic chest discomfort
AC-05	Pulmonary edema; do not give labetalol under AM-04
AO-03	Pregnancy-related hypertension/eclampsia concern

Documentation and release control

- Document focused findings, BP/rhythm/ECG when applicable, all interventions with dose/route/time/response, serial reassessments, destination decision, and notifications.

- This source PDF replaces the legacy imported sheet and matches the approved Claiborne EMS app content reconciled October 16, 2026.

- Use the linked protocol whenever the patient crosses into another clinical pathway. Follow agency release governance, training requirements, and current medication/device availability.